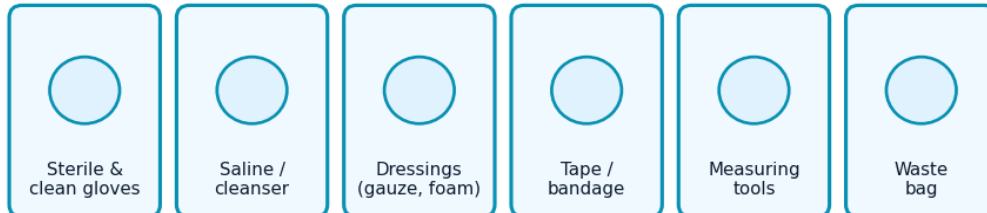


Wound Dressing Change Procedure

Clinical protocol for wound assessment and dressing change

Assemble Equipment (Sterile Field)

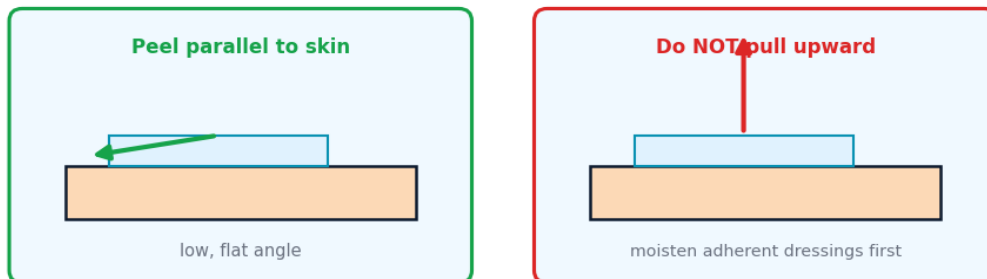


Perform hand hygiene before and after — review the wound care order first.

Step 1: Assess and Gather Equipment

Review the wound care order and last dressing change notes. Gather: sterile gloves, clean gloves, wound cleansing solution (normal saline or prescribed solution), appropriate dressing (gauze, foam, hydrocolloid, alginate per wound type), tape or bandage, wound measurement tools, and a waste bag. Perform hand hygiene.

Removing the Old Dressing

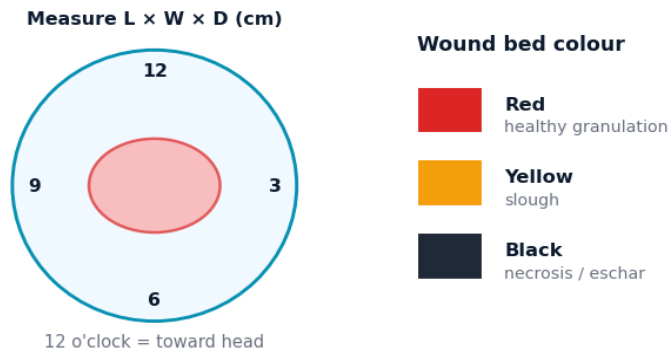


Step 2: Remove the Old Dressing

Don clean gloves. Loosen tape edges carefully — use adhesive remover for skin-friendly removal if needed. Remove the old dressing gently, peeling back parallel to the skin rather than pulling upward. Note drainage amount, color, odor, and consistency. Place soiled dressing in waste bag. Remove gloves and perform hand hygiene.

■ Do not forcibly remove adherent dressings — moisten with saline first to prevent trauma to fragile wound bed.

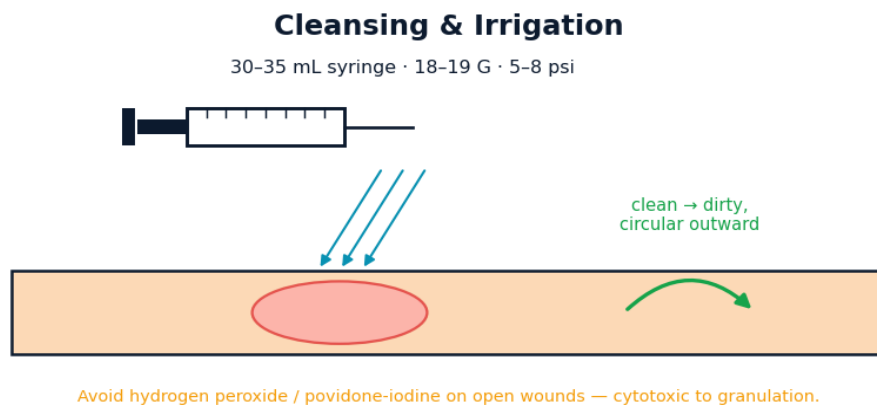
Wound Assessment — Clock Method & Bed Colour



Step 3: Assess the Wound

Using sterile technique, assess: wound dimensions (length × width × depth in cm), wound bed color (red = healthy granulation, yellow = slough, black = necrosis), wound edges (undermining, tunneling), periwound skin condition, exudate level and type. Document all findings.

Note: Measure wound using clock face orientation — 12 o'clock = toward head, 6 o'clock = toward feet.



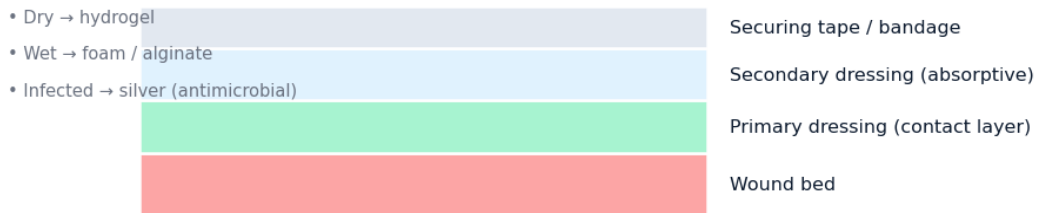
Step 4: Cleanse the Wound

Use prescribed solution (normal saline is most common). Irrigate with gentle pressure (5–8 psi) using a 30–35mL syringe with an 18–19 gauge angiocatheter, or use a commercial wound cleanser. Cleanse from cleanest to dirtiest area in a circular motion moving outward. Pat dry the surrounding skin gently.

■ Do not use hydrogen peroxide or povidone-iodine on open wounds routinely — they are cytotoxic to granulation tissue.

Dressing Application — Layered Cross-Section

Match dressing to wound moisture

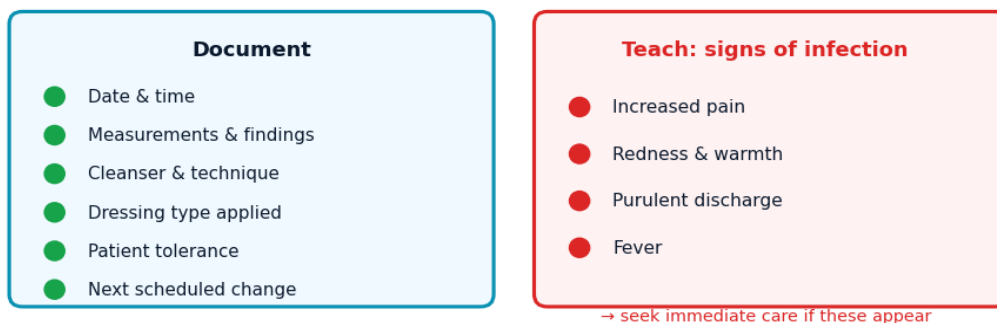


Step 5: Apply the New Dressing

Select dressing appropriate to wound stage: dry wounds → moisture-retentive (hydrogel), wet wounds → absorptive (foam/alginate), infected → antimicrobial (silver). Apply primary dressing directly to wound bed without stretching. Apply secondary dressing if needed. Secure with tape on all edges, avoiding circumferential taping.

Note: Leave dressing in place per order frequency — changing too often disrupts wound healing environment.

Document & Educate Patient



Step 6: Document and Educate

Document: date and time, wound measurements and assessment findings, cleansing solution and technique used, dressing type and materials applied, patient tolerance, next scheduled change. Educate patient on signs of infection (increased pain, redness, warmth, purulent discharge, fever) and when to seek immediate care.